

PATIENT HEALTH FEEDBACK & MANAGEMENT REPORT

Prepared with clinical care and professional integrity

PATIENT DETAILS Name: Mrs Moeniba KamalDIEM Age/Sex: 60 years Female DOB: 10 September 1965 Med Aid: PRIMTRACE Contact: 0827108658	LABORATORY DETAILS Laboratory: PathCare Constantiaberg Medi-Clinic Requisition No: 664106605 Collected: 31 May 2026 at 22:30 Report Generated: 2 June 2026 at 13:48 Referring Doctor: Dr Zahid Badroodien (The Urgent Care Clinic, Plumstead) Referral Diagnosis: I10 — Essential Hypertension
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A MESSAGE FOR YOU, MRS KAMALDIEM

Please read this report calmly and without worry. These results have been carefully reviewed, and most of your tests came back in the normal range. There are one or two areas that simply need a little attention and follow-up care — nothing alarming. You are in good hands, and this report is here to help you understand exactly what is happening with your health and what small steps you can take to feel even better.

SECTION 1: YOUR FULL LABORATORY RESULTS AT A GLANCE

Below are all the tests that were performed on your blood sample, collected on 31 May 2026. The results are explained simply next to each test.

1.1 Sample Quality Check

Test	Your Result	Normal Range	Status
Lipaemic (Fatty blood)	ABSENT	ABSENT	✓ Normal
Icteric (Yellow discolouration)	ABSENT	ABSENT	✓ Normal
Haemolysis (Blood cell damage)	ABSENT	ABSENT	✓ Normal

What this means for you: Your blood sample was of excellent quality. This confirms that all results below are reliable and accurate.

1.2 Electrolytes (Body Salts & Minerals)

Test	Your Result	Normal Range	Status
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S-Sodium (Salt level)	144 mmol/L	136–145 mmol/L	✓ Normal
S-Potassium (K ⁺)	3.4 mmol/L	3.5–5.1 mmol/L	⚠ Low-Normal
S-Chloride	102 mmol/L	101–110 mmol/L	✓ Normal
S-Bicarbonate	27.2 mmol/L	22.0–28.0 mmol/L	✓ Normal
Anion Gap	15 mmol/L	3–15 mmol/L	✓ Upper Normal

What these mean for you: Your body salts are mostly in a good balance. Your potassium is just slightly lower than ideal — this is explained fully in Section 2.

1.3 Kidney Function

Test	Your Result	Normal Range	Status
S-Urea (Kidney waste product)	4.1 mmol/L	2.9–8.2 mmol/L	✓ Normal
S-Creatinine (Kidney filter)	61 µmol/L	49–90 µmol/L	✓ Normal
eGFR (Kidney function %)	95 ml/min/1.73m ²	≥ 90	✓ Excellent (G1)

What these mean for you: Excellent news — your kidneys are working very well. An eGFR of 95 is in the normal range for your age. No kidney damage was detected.

1.4 Cholesterol (Lipogram)

Test	Your Result	Normal Range	Status
Specimen Timing	Not Specified	Fasting preferred	⚠ Note
Lipaemia (Fatty sample)	ABSENT	ABSENT	✓ Normal
S-Cholesterol (Total)	5.9 mmol/L	< 5.0 mmol/L	⚠ Elevated
CKD-EPI eGFR	95 ml/min/1.73m ²	≥ 90	✓ Normal

What these mean for you: Your total cholesterol is slightly above the recommended level. This is a common finding and can often be managed well with diet and, if needed, medication. A full fasting cholesterol test is needed to get the complete picture.

1.5 Thyroid Function (Gland That Controls Your Metabolism)

Test	Your Result	Normal Range	Status
Free T4 (Thyroid hormone)	12.4 pmol/L	9.0–19.1 pmol/L	✓ Normal
S-TSH (Thyroid signal)	1.16 mIU/L	0.35–4.94 mIU/L	✓ Normal
Lab Comment	EUTHYROIDISM	—	✓ Normal Thyroid

What these mean for you: Your thyroid gland is working perfectly. The laboratory itself confirmed this with the comment 'EUTHYROIDISM' — meaning your thyroid is completely normal. No thyroid treatment is needed.

SECTION 2: UNDERSTANDING YOUR POTASSIUM — WHY IT IS LOW

Mrs Kamalidien, please do not be alarmed by your slightly low potassium. The reason for this is almost certainly your blood pressure tablet, and it is something we can address easily.

2.1 What Is Potassium and Why Does It Matter?

Potassium is a mineral in your blood that helps your heart beat regularly, your muscles work properly, and your blood pressure stay stable. Think of it as the body's natural electricity supplier. When potassium is too low, you might feel tired, have muscle cramps, or feel your heart beating oddly.

2.2 Why Is Your Potassium Low? — Three Possible Causes

CAUSE 1: Your Ridaq (Hydrochlorothiazide 12.5mg) Tablet — Most Likely Reason

Ridaq is a diuretic (water tablet). It is excellent for controlling blood pressure because it helps your kidneys remove excess fluid and salt from the body. However, this process unfortunately also washes out potassium through your urine — even at the low dose of 12.5mg that you are currently taking.

Clinical Inquiry: We Need to Know...

1. How long have you been taking Ridaq?
2. Are you taking it every day without missing a dose?
3. Have you been prescribed any potassium supplement alongside it?
4. Have you noticed increased urination since starting Ridaq?

CAUSE 2: Vomiting on the Night of Your Visit — Important Clinical Factor

Your bicarbonate result came back at 27.2 mmol/L — this is at the upper end of normal. When a person vomits, the stomach loses acid, which can gently push bicarbonate levels upward. This pattern in your blood results is subtly consistent with someone who has been vomiting. Vomiting also directly causes potassium loss through the stomach fluid.

Clinical Inquiry: We Need to Know...

1. Did you vomit before or during your visit to the clinic that night?
2. How many times did you vomit, and for how long?
3. Did you feel nauseous at home before coming in?
4. Were you able to eat and drink normally that day?

CAUSE 3: IV Drip (Intravenous Fluids) — Did It Affect the Blood Test?

If a drip was started to rehydrate you before the blood was drawn, this is critically important to know. Intravenous fluids can dilute the potassium in your blood, making it appear lower than it truly is. If dextrose (glucose) fluid was used, it causes potassium to move from the blood into the cells — again making the reading appear falsely low.

If Blood Was Drawn BEFORE the Drip

The result of 3.4 is your true potassium level — reliable and accurate.

If Blood Was Drawn AFTER the Drip

The result may be falsely low. A repeat fasting blood test is recommended to confirm the true level.

Clinical Inquiry: We Need to Know...

1. Was a drip (IV line) inserted during your visit?
2. Were the blood samples taken before or after the drip was connected?
3. What type of fluid was used in the drip (normal saline, Ringers, dextrose)?

SECTION 3: PRACTICAL DIETARY ADVICE TO BOOST YOUR POTASSIUM

The good news is that potassium can be increased naturally through your everyday diet. Here are practical, affordable, South African-friendly food choices to help raise your potassium levels.

Food	Potassium Content	Practical Tip
Banana (1 medium)	~422 mg	<i>Have one with breakfast daily</i>
Sweet potato (1 medium, cooked)	~540 mg	<i>Replace regular potato at dinner</i>
Spinach (½ cup cooked)	~420 mg	<i>Add to stew, eggs, or soup</i>
Avocado (½ fruit)	~487 mg	<i>Spread on toast or add to salad</i>
Beans / Lentils (½ cup cooked)	~365 mg	<i>Add to soup or curry — affordable</i>
Tomato paste / purée (2 tbsp)	~275 mg	<i>Use in all cooking — cheap & easy</i>
Orange / orange juice (1 glass)	~496 mg	<i>Morning drink — fresh is best</i>
Plain yoghurt (1 cup)	~380 mg	<i>With fruit for breakfast or snack</i>
Potato (1 medium, skin on)	~926 mg	<i>Bake or boil — do not fry</i>
Chicken / fish (100g)	~300 mg	<i>Already part of your daily diet</i>

⚠ Foods That Reduce Potassium Absorption — Limit These:

Excess salt / processed foods (chips, polony, canned soups) | I have to mention I am aware of Mrs Kamaldien Religion (Alcohol) | Excessive coffee (more than 2 cups/day) | Sugary fizzy drinks

SECTION 4: YOUR CHOLESTEROL — WHAT IT MEANS AND WHAT TO DO NEXT

Your total cholesterol came back at 5.9 mmol/L. The recommended level is below 5.0 mmol/L. This finding, combined with your diagnosis of high blood pressure and your age, means it is important to take this seriously — but calmly. It is very manageable.

4.1 Why a Full Fasting Lipogram Is Needed

The test that was done only measured your total cholesterol. To get the full picture, a fasting lipogram is required. This means you must not eat or drink anything (except water) for 9 to 12 hours before the blood test. The fasting lipogram will tell us:

- LDL Cholesterol — the 'bad' cholesterol that can clog your arteries
- HDL Cholesterol — the 'good' cholesterol that protects your heart
- Triglycerides — fats in your blood that increase heart risk
- Non-HDL Cholesterol — a combined risk measure

These four values together allow your doctor to calculate your exact heart disease risk and decide whether medication is needed.

4.2 Are You on a Cholesterol Tablet (Statin)? — We Need to Establish This

It is not yet known whether you are currently taking a cholesterol-lowering tablet. This is important because:

- If you are NOT on a statin — the elevated cholesterol may indicate that one should be started after the fasting lipogram
- If you ARE on a statin — the elevated reading may suggest the dose needs adjustment, or that doses have been missed

Questions to Clarify Your Statin Status:

1. Are you currently taking any 'cholesterol tablets'?
2. Do you take any of these: Lipitor, Crestor, Rosuvastatin, Atorvastatin, Simvastatin, or Zocor?
3. Have you ever been told your cholesterol was high before?
4. Have you been prescribed cholesterol tablets before but stopped taking them?
5. Have you had any muscle pains or weakness since taking any prescribed medication?

4.3 Cardiovascular (Heart Attack & Stroke) Risk Assessment

Because you have high blood pressure AND slightly elevated cholesterol, your doctor will calculate your 10-year risk of a heart attack or stroke. To do this accurately, the following questions must be answered at your next visit:

CVD Risk Score Questions for Your Next Appointment:

1. Do you smoke cigarettes, or have you smoked in the past?
2. Has any family member (parent, sibling) had a heart attack or stroke before the age of 60?
3. Do you have diabetes, or have you been told your blood sugar is high?

4. How physically active are you — do you walk, exercise, or do household activities daily?
5. How would you describe your usual diet — lots of fried food, takeaways, or home-cooked meals?
6. How long have you had high blood pressure, and has it been well controlled?

SECTION 5: BLOOD PRESSURE MONITORING — HOW TO DO IT CORRECTLY

Monitoring your blood pressure regularly is one of the most important things you can do for your health. We recommend bi-weekly checks — here is everything you need to know.

5.1 What the Numbers Mean

A blood pressure reading has two numbers, for example: 128/82 mmHg

- The TOP number (systolic) — the pressure when your heart BEATS and pumps blood
- The BOTTOM number (diastolic) — the pressure when your heart RESTS between beats

Category	Reading (mmHg)	Action Required
Optimal	< 120 / 80	Excellent — continue current care
Normal	120–129 / 80–84	Good — maintain lifestyle habits
High Normal	130–139 / 85–89	Monitor carefully — lifestyle review
Grade 1 High BP	140–159 / 90–99	Contact clinic — medication may need adjustment
Grade 2 High BP	160–179 / 100–109	Contact clinic urgently within 24 hours
Grade 3 / Crisis	≥ 180 / ≥ 110	Go to emergency immediately
LOW BP (Hypotension)	< 100 / 60	Sit or lie down — contact doctor if dizzy/faint

5.2 How to Take Your Blood Pressure Correctly — Step by Step

1. Sit quietly for 5 minutes before measuring — do not measure after walking, exercising, or being stressed
2. Sit with your back supported, feet flat on the floor — do not cross your legs
3. Rest your arm on a flat surface at heart level — same arm every time (preferably left arm)
4. Do not talk or use your phone while measuring
5. Take two readings, 2 minutes apart — record the average
6. Measure at the same time each day — morning (before tablets) and evening are ideal
7. Write down every reading in a diary with the date and time — bring to every appointment

5.3 Monitoring Frequency Schedule

Situation	Recommended Frequency
BP well controlled (< 130/80)	Bi-weekly — every two weeks
BP slightly elevated (130–139/85–89)	Weekly monitoring — review at next appointment
BP high (≥ 140/90)	Twice weekly — contact clinic within 1 week
BP very high (≥ 160/100)	Daily — contact clinic urgently

BP low (< 100/60) with symptoms	Immediate contact with doctor — sit/lie down
After any medication change	Daily for 2 weeks, then return to bi-weekly

5.4 Danger Symptoms — Go to Emergency Immediately If You Experience:

Sudden severe headache | Blurred or double vision | Chest pain or tightness | Sudden arm/face/leg weakness | Difficulty speaking | Shortness of breath at rest | Rapid or irregular heartbeat

SECTION 6: YOUR MEDICATION — WHY TAKING IT EVERY DAY MATTERS

Mrs Kamaldiem, this section is very important. High blood pressure is called the 'silent killer' because it causes no symptoms — you will often feel perfectly fine even when your blood pressure is dangerously high. This is exactly why taking your tablet every day, even when you feel well, is so critical.

6.1 Your Current Medication: Ridaq 12.5mg (Hydrochlorothiazide)

Ridaq is a thiazide diuretic (water tablet). It works by helping your kidneys remove excess fluid and salt from your body, which reduces the pressure inside your blood vessels. It is a proven, effective, first-line blood pressure medication.

6.2 What Happens If You Miss Doses?

When You Take It Every Day:

Blood pressure stays controlled, protecting your heart, kidneys, brain, and eyes consistently.

When You Miss Doses:

Blood pressure rises silently. Risk of stroke, heart attack, and kidney damage increases — even if you feel fine.

6.3 Practical Medication Adherence Tips

- Take Ridaq at the same time every morning — link it to brushing your teeth or having breakfast
- Take it in the MORNING — not at night. It causes urination and you do not want to be waking up at 2am
- Set a daily phone alarm labelled 'My heart tablet' as a reminder
- Keep your tablets visible — do not hide them in a drawer
- Never stop suddenly without talking to your doctor — blood pressure can spike dangerously
- Get your refill prescription before your last tablet — do not wait until you run out
- Tell your doctor if you are experiencing any side effects — there are alternatives available

6.4 Common Side Effects of Ridaq — When to Be Concerned

Side Effect	What To Do
Increased urination (especially first few weeks)	<i>This is expected and will settle — keep taking the tablet</i>
Dizziness when standing up quickly	<i>Rise slowly — sit on the edge of the bed first before standing</i>
Muscle cramps or weakness	<i>Report to your doctor — may indicate low potassium</i>
Sensitivity to sunlight	<i>Use sunscreen and wear a hat outdoors</i>
Dry mouth or thirst	<i>Drink water regularly throughout the day</i>

SECTION 7: YOUR FOLLOW-UP PLAN — WHAT HAPPENS NEXT

Priority	Action	Why It Is Needed	Timeframe
● Urgent	Clarify blood sampling vs drip timing	<i>Determines if potassium result is reliable</i>	Now — ask your doctor
● Urgent	Confirm statin medication status	<i>Establish if cholesterol treatment is in place</i>	Next appointment
● Urgent	Fasting full lipogram (LDL, HDL, TG)	<i>Full heart risk assessment</i>	Within 2 weeks (fast 10 hrs)
△ Soon	Repeat U&E + electrolytes	<i>Confirm potassium after dietary & medication review</i>	In 4–6 weeks
△ Soon	10-year CVD risk score calculation	<i>Decide if statin is needed</i>	At fasting lipogram visit
📅 Ongoing	Bi-weekly BP monitoring	<i>Track blood pressure control</i>	Ongoing
📅 Ongoing	Increase dietary potassium daily	<i>Natural correction of low potassium</i>	Starting today
📅 Ongoing	Daily Ridaq adherence	<i>Prevent BP spikes and organ damage</i>	Every morning

A FINAL WORD FOR YOU, MRS KAMALDIEM

Please know that these results, taken as a whole, tell the story of a person whose body is doing a remarkable job. Your kidneys are healthy, your thyroid is normal, and the two areas that need attention — your potassium and your cholesterol — are both very manageable. You are not in danger. You are simply being monitored, and that is exactly the right thing to do.

Take your tablet every morning, eat well, monitor your blood pressure, and attend your follow-up appointment. That is all that is being asked of you. You have got this.

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Ethical Pledge Statement: *While my formal membership designation with the Compliance Institute of Southern Africa (CISA) is pending completion of the final certification modules, I have formally pledged to adhere to the Code of Ethical and Professional Conduct. This commitment informs my clinical and forensic approach to this report, ensuring that all findings are presented with the integrity, independence, and professional care required of the compliance profession.*

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